38yr, male, car crash-multisystem trauma

Scenario Set Up	*Car crash, bystander/pedestrian on the street called 911. The patient is altered in their car seat. The airbags have gone off. There is no vehicular intrusion.*
Dispatch	*38yr male, single vehicle car crash on the freeway*
Scene Size Up	*Active freeway, must shield with ambulance, stabilize spine (KED + Backboard)*
Pertinent Primary Assessment Findings	*AVPU - A&O X 2 (dont know where they are/what happened/struggle with time)* *X - No major bleeding (minor bleeding from nose)* *A - airway patent* *B - Paradoxical breathing (diminished lung sounds* *over damaged area). Obvious respiratory distress and shallow breathing.* *C - skin: pale, cool, moist; pulse: weak (bradycardic)*
Pertinent Secondary Assessment Findings	Head-toe assessment: ● Head: battle's signs, raccoon eyes, CSF and bleeding in ears, bruising on face, bloody nose ● Chest: Flail chest, bruising ● Back: Cervical spinal fracture/deformity (found when palpating spine) ● Rest is normal Patient cannot answer SAMPLE, OPQRST due to altered mental status and poor breathing.
Vitals	*Initial: BP: 90/45, HR: 55, RR: 30 (10 with BVM), BGL: 90, SPO2: 92% (94% after O2) , E: PERRL intact* *Second set: BP: 86/44, HR: 50, RR: 10 with BVM, SPO2: 94%, E: PERRL intact*
Treatments	*O2, BVM; spinal immobilization, shock treatment (supine, O2, warm, fast transport)*
Key Points	- *The patient was in neurogenic shock.* - *Ensure to check for cushing's triad (Intracranial Pressure) from potential subdural/epidural hematomas with head injuries*

	- *Ensure to check for anterograde / retrograde amnesia with head injuries.* - *Check the back of the body before strapping to the backboard.* - *Continuously assess mental status.*
Bonus Questions	*How often do we check on this patient?* *What type of shock?*

18yr, female, hurt ankle

Scenario Set Up	*Patient fell while rock climbing and sprained ankle badly. Patient can't stand on ankle, and is lightheaded when sitting up.*
Dispatch	*18yr female complaining of ankle injury4*
Scene Size Up	*Patient laying on the ground holding leg, elevated ankle.*
Pertinent Primary Assessment Findings	*AVPU - How alert is the patient - indicate which part of AVPU they are? A&O X 4 (Indicate what they are A&O to)* *A - airway patent* *B - breathing rapid, regular, normal* *C - skin pale, warm, dry; pulse rapid, regular, strong*
Pertinent Secondary Assessment Findings	*Indicate here the patient/proctors answers to SAMPLE questions, along with any pertinent further assessments (focused or complete physical assessments, AEIOUTIPS, OPQRST, etc.)* S- patient in a lot of pain. Ankle swollen. Light headed when they stand up/sit up. A - *no allergies* M - *no medication* P - *no* L - *proctor's choice* E - *was rock climbing when slipped from the top and fell directly onto ankle. Ankle snapped sideways* *O - immediately after fall* *P - laying down helps light headedness/sitting up makes it worse* *Q - throbbing pain* *R - doesn't radiate* *S - 7/10 but pain is getting better* *T - Hurting since fall*
Vitals	*Indicate the patient's vital signs-> add a second set if necessary for after initial treatment* *BP: 122/94, HR: 85, RR: 18, BGL: 85, SPO2: 96% , Any other relevant vitals*

Treatments	*Splint and transport, nothing life threatening (stabilize manually while asking questions/before splinting)*
Key Points	*Check CMS before and after splinting*
Bonus Questions	*Opt. patient wants pain medication, what should you say?*

24-Year-Old Male – Stab Wound to Right Thigh

Scenario Set Up	Patient was stabbed in the right thigh during an altercation. He is sitting on the sidewalk with significant bleeding, complaining of pain and dizziness.
Dispatch	*24-year-old male with stab wound to right leg.*
Scene Size Up	*Ensure scene is secured by law enforcement. PPE, one patient, request ALS.*
Pertinent Primary Assessment Findings	*LOC: Alert, A&O × 3 (confused about time), GCS 14. Mild* *AMS. X: Severe bleeding from right thigh. A: Airway patent. B:* *Rapid, shallow breathing (RR 26). C: Pale, cool, clammy skin;* *rapid, weak pulse. D: Mild confusion, follows commands. E:* *Expose injury and prevent hypothermia.*
Pertinent Secondary Assessment Findings	*Head-to-toe: Head/neck unremarkable.* *Chest rise equal, lungs clear.* *Abdomen soft, non-tender.* *Pelvis without obvious injury.* *Upper extremities unremarkable.* *Right thigh has deep penetrating wound with severe bleeding.* *Back without visible injuries.* *Assess distal CMS. SAMPLE: Severe thigh pain, dizziness; NKDA; no medications or medical history; ate 3 hours ago; stabbed 10 minutes ago.*

	OPQRST: Worse with movement, slightly relieved by rest, sharp pain, non-radiating, 9/10, constant since injury.
Vitals	*BP: 88/54, HR: 132, RR: 26, SpO_2: 96%, BGL: 105, GCS: 14.*
Treatments	*Control severe bleeding with a tourniquet if feasible; use hemostatic wound packing and direct pressure if the wound is too proximal.* *Reassess bleeding, treat for shock, prevent hypothermia, request ALS, and rapidly transport to a trauma center.* *Monitor oxygenation and provide oxygen if indicated.*
Key Points	*Recognize hemorrhagic shock, prioritize XABCD, identify mild AMS, perform a rapid head-to-toe assessment, and distinguish tourniquet use from wound packing.*
Bonus Questions	*1. What is the patient's LOC?* *2. What indicates hemorrhagic shock?* *3. When should wound packing be used instead of a tourniquet?* *4. Why perform a rapid head-to-toe assessment?*